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Building Your TMS Referral Network

Turn Colleagues Into Your Best Lead Source — A Practical Guide to Physician Referral Development for TMS Clinics

- ✓ Identify and prioritize referral targets across 6 specialty categories
- ✓ 5-touch outreach campaign with email, letter, and LinkedIn templates
- ✓ Lunch-and-learn playbook with presentation outline and talking points
- ✓ Referral tracking spreadsheet and KPI framework
- ✓ Co-management agreement and quarterly report templates

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Why Referral Networks Matter for TMS

3–5×	$38	68%	12mo
higher conversion vs. cold leads	avg cost per referral patient	of TMS patients seek physician guidance first	typical physician referral cycle

> *Our referral network now accounts for 60% of new patient volume. The lunch-and-learn program was the single highest-ROI investment we made in our first year — $400 in catering generated $180,000 in billed sessions.*
>
> **Dr. Rachel Mendez, MD**
> Founder, Clarity TMS Center, Phoenix, AZ

> *We built a spreadsheet to track every referral source, and within 6 months we could clearly see which specialties sent patients who actually completed treatment. We doubled down on psychiatrists and drastically cut our outreach to neurologists who were referring but whose patients never scheduled.*
>
> **Kenji Watanabe**
> Practice Manager, Pacific TMS Institute, Seattle, WA

1. Mapping Your Referral Landscape

Before you can build a referral network, you need to know who to build it with. Not all referral sources are equal — the best partners are physicians whose patient populations overlap meaningfully with TMS candidates: treatment-resistant depression, anxiety disorders, OCD, and comorbid conditions.

Target Specialty Categories

Specialty	Typical Patient Overlap	Referral Volume Potential	Outreach Priority
Adult Psychiatrists	Highest — manage TRD patients directly, often have already tried multiple medications	High (8–15 referrals/practice/year)	HIGHEST
Primary Care / Internal Medicine	High — first point of contact for depression, can screen for TMS eligibility	Medium–High (5–10/practice/year)	HIGHEST
Psychiatric NPs / PAs	High — prescribe and manage TMS under supervision, frequent patient contact	Medium (4–8/practice/year)	HIGH
Psychologists / Therapists	Medium — see patients who are already in treatment, can identify medication failures	Medium (3–6/practice/year)	HIGH
Neurologists	Medium — some overlap (depression with neurological comorbidities)	Low–Medium (2–4/practice/year)	HIGH
Pain Specialists / Physiatrists	Low — limited depression overlap, but may refer for comorbid conditions	Low (1–3/practice/year)	MODERATE

Finding Referral Targets in Your Area

DIGITAL RESEARCH

- Search "psychiatrist near me accepting new patients" — note which practices appear
- Check hospital medical staff directories — lists all affiliated physicians by specialty
- Review your own patient intake forms — ask "who referred you?" to identify existing referral patterns
- Google "TMS therapy [city]" — see which clinics come up and who they cite

NETWORK INTELLIGENCE

- Ask your medical director to reach out to their residency/fellowship contacts
- Attend your local medical society meetings (typically monthly)
- Connect with referring physicians on LinkedIn — see which groups they belong to
- Ask hospital discharge planners for psychiatry contact lists

> **GOLDEN RULE OF REFERRAL MAPPING**
>
> Start with your top 10 referral targets — the ones who could realistically send you 5+ patients per year. A focused network of 20 engaged referring physicians will outperform a superficial list of 200 barely-contacted names. Quality of relationship matters more than quantity of contacts.

2. The Outreach Campaign

Cold outreach to physicians requires persistence and professionalism. The research on physician outreach shows that 80% of referrals come after the 3rd+ contact. This section provides the templates and cadence for a structured 5-touch campaign.

5-Touch Outreach Sequence

1. **Touch 1 — Email (Week 1):** Personalized cold email introducing your clinic, TMS, and what makes your practice unique. Keep to 5 sentences. Attach a one-page TMS overview PDF.
2. **Touch 2 — Phone Call (Week 2):** Call their office, ask to speak with the physician or their medical assistant. Leave a voicemail if needed. Reference your email by name. Script: "I'm reaching out following an email I sent about TMS therapy at [Clinic]. I'd love 5 minutes to discuss whether your practice has patients who might benefit."
3. **Touch 3 — Formal Letter (Week 3):** A printed letter on clinic letterhead sent via USPS to the physician's office address. More likely to reach the physician directly than email. Include a stamped self-addressed reply card.
4. **Touch 4 — LinkedIn Connection (Week 5):** Send a personalized LinkedIn connection request referencing your letter or call. Once connected, send a brief message. LinkedIn is effective because physicians check it regularly and it feels less "sales-y" than cold email.
5. **Touch 5 — Second Call (Week 7):** Final call attempt. If they answer, offer to schedule a 10-minute call at their convenience. If they decline, ask to be added to their newsletter or to receive referrals if the right patient comes up.

Cold Outreach Email Template

EMAIL TEMPLATE — COLD OUTREACH

Subject: TMS for Treatment-Resistant Depression — Partnership Inquiry

Dear Dr. [*LAST NAME*]:

I hope this finds you well. I'm [*YOUR NAME*], [*CLINIC TITLE*] at [*CLINIC NAME*] in [*CITY*].

We specialize in Transcranial Magnetic Stimulation (TMS) — an FDA-cleared, non-invasive treatment for treatment-resistant depression that's proving transformative for patients who have not responded to multiple medication trials.

I'd love to explore whether we might be a helpful resource for your patients. Specifically:

- Patients with major depressive disorder who have tried 2+ medications without adequate response
- Patients experiencing medication side effects that limit treatment options
- Patients who prefer non-pharmacological treatment options

I've attached a brief one-pager on TMS and our practice. Would you be open to a brief call — even 10 minutes — to discuss whether a referral relationship makes sense?

Warm regards,

[*YOUR NAME*]
[*CLINIC NAME*]
[*PHONE*] | [*EMAIL*]
[*CLINIC WEBSITE*]

Formal Letter Template

USPS LETTER TEMPLATE

[*CLINIC LETTERHEAD*]

DATE

Dr. [*NAME*]
[*PRACTICE NAME*]
[*STREET ADDRESS*]
[*CITY, STATE ZIP*]

Dear Dr. [*LAST NAME*]:

I'm writing to introduce you to [*CLINIC NAME*], a TMS therapy center serving patients in the [*REGION*] area.

Transcranial Magnetic Stimulation is FDA-cleared for treatment-resistant major depression and has helped thousands of patients who have not found relief through medication alone. For the right patients — those who have tried 2 or more antidepressants without adequate response — TMS can be life-changing.

We believe the best patient care happens through physician collaboration. We would be honored to serve as a resource for your practice.

Enclosed is a brief overview of our clinic and our approach. Please use the reply card to let us know if you'd like to be added to our referral list, or feel free to call or email directly.

Warm regards,

[*SIGNATURE*]
[*NAME, CREDENTIALS*]

Enclosure: TMS Clinic Overview (1 page)
Stamped Self-Addressed Reply Card

LinkedIn Connection Template

LINKEDIN CONNECTION REQUEST MESSAGE

Hi Dr. [*LAST NAME*],

I recently sent you information about our TMS clinic in [*CITY*] and wanted to connect here as well.

TMS (transcranial magnetic stimulation) is an FDA-cleared treatment for treatment-resistant depression — something I know comes up in psychiatric practice regularly.

If you ever have a patient who might be a good fit, I'd welcome the chance to be a trusted referral partner.

Best,
[*YOUR NAME*]
[*CLINIC*]

3. Lunch-and-Learn Playbook

The lunch-and-learn (also called a "lunch-and-learn" or "dine-and-discuss") is one of the most effective physician outreach tactics because it removes the biggest barrier — time. You bring the food; the physician gives you 45 minutes of attention. Done well, a single lunch-and-learn can generate 3–8 referrals within 3 months.

Planning Checklist

Task	Details	Done
Set date & time	Mid-week, 12:00–1:00 PM works best. Confirm with physician's MA 2 weeks in advance.	☐
Secure venue	Physician's office conference room, your clinic, or a nearby private dining room. Avoid loud restaurants.	☐
Order catering	$150–$250 budget. Order from a well-regarded local restaurant. Avoid pizza. Send order confirmation to physician's office 1 week prior.	☐
Prepare materials	One-page TMS overview, referral form with your fax/email, your business card, patient education brochures (3–5 copies)	☐
Prepare presentation	10–12 slides. See outline below. Practice delivering in under 30 minutes — leave 15 min for Q&A.	☐
Send reminder	Email physician 2 days before confirming time, location, and parking instructions.	☐
Follow-up	Email thank-you within 24 hours with PDF of presentation, referral fax form, and your contact info.	☐

20-Slide Presentation Outline

SLIDES 1–5: INTRODUCTION

- Title slide: Your clinic name, your name/title
- Who we are: clinic overview, locations, years in operation
- What is TMS: 2-slide technical overview with mechanism of action (non-invasive, magnetic pulses, prefrontal cortex target)
- FDA clearance: 2008 for NeuroStar, insurance coverage overview

SLIDES 6–10: CLINICAL EVIDENCE

- Clinical trial data: 50+ RCTs, remission rates 28–32%
- Comparison with medications: side effect profile advantage
- Who is a candidate: TRD criteria, PHQ-9 thresholds
- Who is NOT a candidate: contraindications
- Outcomes at our clinic: aggregate response/remission data

SLIDES 11–15: THE REFERRAL PROCESS

- How to refer: fax, email, phone — show the referral form
- What happens after referral: insurance verification, consultation scheduling
- Communication loop: how and when we update referring physician
- Timeline: from referral to first treatment session
- Ongoing care: maintenance sessions, discharge summary

SLIDES 16–18: Q&A PREP

- Anticipated objections and responses table
- Case study: 1–2 de-identified patient success stories
- Resources: referral form, contact card, patient brochures

SLIDES 19–20: CLOSE

- "Who should you send?" — reminder of ideal candidate criteria
- Contact information — make this slide memorable, high contrast

Anticipated Questions & Responses

Question	Suggested Response
"Is TMS covered by insurance?"	Yes — Medicare covers TMS under NCD, and most commercial payers (BCBS, Aetna, Cigna, UHC) cover it with prior authorization. We handle all authorizations.
"How long does treatment take?"	36 sessions over 6–9 weeks (5 sessions/week). Each session is 18–37 minutes. Patients can return to normal activities immediately.
"What if my patient relapses?"	We monitor for relapse and offer maintenance sessions. Most patients who respond to initial TMS also respond to retreatment if needed.
"Do you require a referral form or can patients self-refer?"	We accept both physician referrals and self-referrals, but physician referrals help with insurance authorization and clinical handoff.

4. Referral Tracking Spreadsheet Layout

You cannot manage what you do not measure. Build a referral tracking system from day one — even a simple spreadsheet beats nothing. This section provides the exact column structure for a Google Sheets or Excel tracker.

Tab 1: Referral Source Master List

Provider Name	Specialty	Practice	Contact	Initial Outreach	Last Contact	Referrals Sent	Referrals Converted	Notes	Next Action
Dr. Sarah Chen	Psychiatry	Bay Area Psychiatry	schen@bap.com | (415) 555-0102	Jan 15, 2025	Mar 3, 2026	11	8	Lunch-and-learn Jan 2025. Excellent relationship. Refers patients with comorbid anxiety well.	Send quarterly report Mar 2026
Dr. Michael Torres	Internal Medicine	Memorial Health PCP	mtorres@mhpcp.org | (312) 555-0174	Apr 2, 2025	Feb 18, 2026	4	3	Very responsive to faxed materials. Prefers fax referral to email.	Call to schedule lunch Q2 2026
Jennifer Park, NP	Psychiatric NP	Compass Mental Health	jpnp@compassmh.com | (206) 555-0198	Jun 10, 2025	Jan 5, 2026	2	1	Sent initial email and brochure. No response to follow-up. Consider LinkedIn outreach.	LinkedIn connection + follow-up email
Dr. Amanda Singh	Psychiatry	Singh Psychiatry Group	asingh@spgroup.med | (713) 555-0141	Sep 22, 2025	Nov 30, 2025	0	0	Met at Houston Medical Society mixer. Sent letter + email. No response yet.	Second letter + call Week 3

Tab 2: Monthly Referral Log

Date	Patient Name (Last, First)	Referral Source	Referral Method	Diagnosis	Initial PHQ-9	Consult Scheduled?	Treatment Started?	Completion Status	Notes
Jan 8, 2026	Williams, David	Dr. Sarah Chen	Fax	MDD, TRD	22	Jan 15	Jan 22	Completed (36 sessions)	Remission achieved, PHQ-9 at 4

Date	Patient Name (Last, First)	Referral Source	Referral Method	Diagnosis	Initial PHQ-9	Consult Scheduled?	Treatment Started?	Completion Status	Notes
Jan 14, 2026	Patel, Ananya	Dr. Michael Torres	Phone	MDD, GAD	18	Jan 22	Feb 3	In progress (Session 22)	Concurrent GAD responding well
Feb 2, 2026	Okafor, James	Self-referral (web)	Website form	MDD, TRD	25	Feb 8	—	No-show, consult rescheduled	Insurance verified, auth pending

KPI TARGETS FOR REFERRAL PROGRAM

Track these monthly: (1) Referrals sent per active referring physician — target ≥1/month/physician. (2) Referral-to-consult conversion — target ≥70%. (3) Consult-to-start conversion — target ≥55%. (4) Revenue per referral — target ≥$5,000. Any referring physician who hasn't sent a patient in 6 months should get a re-engagement touch.

5. Co-Management Agreement Template

A formal co-management agreement clarifies roles, responsibilities, and communication expectations between your TMS clinic and referring physicians. While not required for every referral, agreements are especially valuable for high-volume referral relationships and institutional partnerships.

CO-MANAGEMENT AGREEMENT — TEMPLATE

AGREEMENT FOR CO-MANAGEMENT OF TRANSCRANIAL MAGNETIC STIMULATION (TMS) THERAPY

This Agreement is entered into as of [*DATE*], by and between:

PARTY A: [*TMS CLINIC NAME*], located at [*ADDRESS*] ("TMS Provider")

PARTY B: [*REFERRING PRACTICE NAME*], located at [*ADDRESS*] ("Referring Physician")

1. PURPOSE. This Agreement outlines the terms under which the Referring Physician will refer patients to the TMS Provider for evaluation and treatment with Transcranial Magnetic Stimulation (TMS) therapy.

2. SCOPE OF CARE — TMS PROVIDER RESPONSIBILITIES.

a. Conduct clinical evaluation and determine TMS candidacy.
b. Obtain prior authorization from payer.
c. Administer TMS treatment per FDA-approved protocols.
d. Monitor patient progress using validated outcome measures (PHQ-9, GAD-7).
e. Communicate treatment outcomes to Referring Physician at baseline, midpoint (Session 18), and completion.
f. Report any adverse events per FDA requirements.
g. Coordinate with Referring Physician regarding concurrent medications.

3. SCOPE OF CARE — REFERRING PHYSICIAN RESPONSIBILITIES.

a. Provide relevant clinical history, including medication trials and psychiatric evaluations.
b. Maintain primary responsibility for psychotropic medication management during TMS course.
c. Be available for consultation if clinical concerns arise during treatment.
d. Respond to communication from TMS Provider within [*2 BUSINESS DAYS*].

4. COMMUNICATION PROTOCOL.

a. Referring Physician will send referrals via: [*FAX / SECURE EMAIL / PORTAL*].
b. TMS Provider will send outcome reports within [*5 BUSINESS DAYS*] of each milestone.
c. Interim reports will be provided upon clinical concern or patient request.

5. TREATMENT RESPONSIBILITY MATRIX.

Decision	TMS Provider	Referring Physician
TMS candidacy	PRIMARY	Consulted
Medication changes	Advised	PRIMARY
TMS protocol adjust.	PRIMARY	Informed
Adverse event mgmt.	PRIMARY	Immediate notify
Discharge planning	PRIMARY	Coordinated

6. LIABILITY. Each party shall maintain professional liability insurance covering their respective scope of practice. Neither party shall be liable for the other's clinical decisions outside their defined scope.

7. TERM & TERMINATION. This Agreement is effective as of [*DATE*] and shall remain in effect for [*1 YEAR*] unless terminated by either party with [*30 DAYS*] written notice.

TMS PROVIDER SIGNATURE: _____ Date: _____
REFERRING PHYSICIAN SIGNATURE: _____ Date: _____

6. Quarterly Referral Report Template

Sending referring physicians a quarterly update is a high-value touchpoint that keeps your clinic top of mind and demonstrates professionalism. It takes 15 minutes to prepare and can reignite a dormant referral relationship.

QUARTERLY REFERRAL REPORT — TEMPLATE

SUBJECT: Your TMS Referral Report — Q[*1/2/3/4*] [*YEAR*]

Dear Dr. [*LAST NAME*]:

Thank you for trusting [*CLINIC NAME*] with your patients this quarter. Below is a summary of the referrals we received from your practice and their outcomes.

REFERRALS FROM YOUR PRACTICE — Q[*NUMBER*]

- Total referrals received: [*NUMBER*]
- Consultations scheduled: [*NUMBER*]
- Treatment courses initiated: [*NUMBER*]
- Patients currently in treatment: [*NUMBER*]

PATIENT OUTCOMES (Aggregate)

- Average baseline PHQ-9: [*SCORE*]
- Average end-of-treatment PHQ-9: [*SCORE*]
- Response rate (≥50% improvement): [*X%*]
- Remission rate (PHQ-9 <5): [*X%*]

PRACTICE UPDATES

- [*NEW PROTOCOL, NEW DEVICE, NEW PROVIDER, NEW LOCATION, ETC.*]

HOW TO MAKE A REFERRAL

Fax: [*FAX NUMBER*]
Secure Email: [*EMAIL*]
Online Portal: [*URL*]
Phone: [*PHONE*]

Thank you again for your partnership in caring for patients with treatment-resistant depression.

[*SIGNATURE*]
[*NAME, CREDENTIALS*]

Quarterly Outreach Cadence

Referring Physician Status	Q1 (Jan–Mar)	Q2 (Apr–Jun)	Q3 (Jul–Sep)	Q4 (Oct–Dec)
Champion (5+ referrals/year)	Quarterly report + holiday card	Quarterly report	Quarterly report	Quarterly report + lunch
Active (1–4 referrals/year)	Quarterly report	Lunch-and-learn offer	Quarterly report	Year-end summary
Dormant (0 referrals, 6+ mo)	Re-engagement email	Letter + call	LinkedIn touch	Quarterly report
New Contact (not yet referred)	Initial outreach sequence	Check-in call	Lunch offer	Intro letter

7. Scaling Your Referral Network

Once you have a core group of referring physicians, it's time to systematize and scale. This section outlines the pipeline stages, KPIs, and expansion strategies for growing from 5 to 50+ active referral sources.

Referral Pipeline Stages

1
Identifying — finding targets in your market

2
Outreach — 5-touch campaign in progress

3
Engaged — responded, met, or expressed interest

4
Active — sent at least 1 referral

Network KPI Dashboard

KPI	Target	Current	Status
Total referring physicians in network	30 by EOY	—	
Active referrers (sent ≥1 in 90 days)	≥50% of network		
Referrals per active referrer per quarter	≥1.0		
Referral-to-consult conversion rate	≥70%		
Consult-to-start conversion rate	≥55%		
Revenue per referral	≥$5,000		
Average time from referral to first treatment	≤14 days		

Expansion Strategies

- **Hospital grand rounds:** Offer to give a 30-minute grand rounds presentation on TMS at local hospitals. This reaches 20–40 physicians in a single session and establishes clinical credibility.
- **State psychiatric association membership:** Join your state psychiatric association and attend annual meetings. These are goldmines for referral relationships.
- **Residency/fellowship partnerships:** Partner with psychiatry residency programs to give TMS didactics. Residents become future referring physicians.
- **Community mental health centers:** FQHCs and community mental health centers serve patients who are often uninsured or underinsured. Build relationships with their psychiatric NPs and psychiatrists.
- **Corporate EAP partnerships:** Employee Assistance Programs can refer employees with depression for TMS as an alternative to medication.

> **THE REFERRAL CHAMPION STRATEGY**
>
> Identify your top 3 referral sources and invest disproportionately in those relationships. Champions are physicians who refer consistently, respond to your communications, and will advocate for you to their colleagues. Give Champions: priority scheduling for their patients, direct phone access to your medical director, quarterly face-to-face meetings, and public acknowledgment (e.g., a "Referral Partner of the Year" certificate).

8. Glossary of Key Terms

Co-management A formal or informal arrangement between two physicians to share care responsibilities for a patient. In TMS, typically between the referring psychiatrist and the TMS clinic physician.

Coordination of Care The deliberate organization of patient care activities between two or more participants involved in a patient's care to facilitate the appropriate delivery of healthcare services.

FQHC Federally Qualified Health Center. A federally-funded community health center that provides primary care regardless of ability to pay. Often serves uninsured and Medicaid patients.

HIPAA Health Insurance Portability and Accountability Act. Federal law protecting patient health information. Applies to all communications between TMS clinics and referring physicians.

Referral Pipeline The stages a referral source moves through from first contact to active referring. Stages: identifying → outreach → engaged → active → champion.

SOAP Note Subjective, Objective, Assessment, Plan. Standard medical documentation format. Used in referral communications to convey patient clinical status.

Transfer of Care The formal process of shifting primary responsibility for a patient's care from one physician to another. Distinct from co-management, which shares responsibility.

Continuum of Care The delivery of healthcare across multiple settings and providers over time. TMS fits within a continuum that includes primary care, psychiatry, psychotherapy, and maintenance care.

EMR / EHR Electronic Medical Record / Electronic Health Record. Digital version of a patient's medical history. HIPAA requires BAAs with all EMR vendors.

Gatekeeper Model A healthcare delivery model in which a primary care physician controls access to specialist services. In TMS, the referring physician or PCP often acts as the gatekeeper.

Referral Conversion Rate The percentage of referrals received that convert to scheduled consultations. Industry benchmark: 60–80%. Low conversion indicates referral friction or patient motivation issues.

Scope of Care The set of services, treatments, and procedures that a healthcare provider is qualified and authorized to perform. Defines what each party can do in a co-management arrangement.

Super-Biller A medical billing specialist with expertise in maximizing reimbursement through correct code selection, modifier usage, and payer-specific rules. TMS clinics benefit from super-billers on staff.

Value-Based Care A healthcare payment model where providers are reimbursed based on patient outcomes rather than volume of services. TMS outcomes data supports value-based contracting with payers.

QR Resource Center

Scan to Connect

Access TMS List resources on any device — no typing required.

tmslist.com/map/

Find a TMS provider near you

tmslist.com/quiz/

Take the TMS eligibility quiz

Tear-Out Card: Referral Network Launch Checklist

TMS Referral Network — Quick-Start Checklist

1. Identify top 10 referral targets by specialty and geography
2. Research each target: practice name, address, contact info
3. Build referral tracking spreadsheet with all 10 targets
4. Customize email template for each physician
5. Send Touch 1 emails to all 10 targets
6. Follow up with Touch 2 phone calls 1 week later
7. Send Touch 3 formal letters via USPS
8. Connect on LinkedIn (Touch 4) for responders
9. Schedule first lunch-and-learn with interested physician
10. Create referral form and fax cover sheet
11. Prepare TMS one-pager PDF for outreach packets
12. Log all outreach in referral tracking spreadsheet
13. Confirm lunch-and-learn catering 1 week prior
14. Deliver lunch-and-learn presentation
15. Send thank-you email with presentation PDF within 24 hours
16. Track first referrals received and conversion
17. Send first quarterly referral report
18. Identify and nurture top referral champion

Get the full playbook at **tmslist.com** — Scan to find a TMS provider near you

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